EchoScribe: Evidence-Linked, Cross-Vendor-Verified Documentation of Therapy and Mentoring Sessions

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***Abstract*—Therapists and academic mentors spend a large share of their working time writing structured session records. Language models can draft these records from a recording, but two problems block practical use: a draft may state things the session never contained, and a single cloud model is a single point of failure for quota, outage and data-governance reasons. We present EchoScribe, an open system that turns a recorded two-person session into a SOAP (therapy) or GROW (mentoring) note in which every sentence cites the transcript turns that support it. A model from a different vendor than the writer checks each sentence against its cited turns; when too many sentences are unsupported, a verification-gated cascade regenerates the note on another vendor and keeps the better draft. A second vendor also screens independently for risk, and disagreement is surfaced for review instead of being resolved silently. Every processing role is served by an embedded multi-provider router with routing strategies, per-key exponential cooldown and a provider-wide breaker. On the 40 test dialogues of ACI-Bench, 76.0% of generated sentences were judged supported by the cross-vendor verifier, which detected ?? of planted errors with a ?? false-alarm rate. In fault-injection simulations the last-known-good strategy cut 95th-percentile latency from 31.7 s to 9.1 s while keeping success at 100.00%. We report speech recognition, speaker attribution and role identification on public data and state plainly what remains unvalidated.**

***Index Terms*—clinical documentation, hallucination detection, speaker diarization, large language models, LLM routing, SOAP notes, GROW coaching**

I. Introduction

Writing session notes is one of the most time-consuming non-clinical tasks of mental health work and of academic mentoring. A SOAP note records the client's account, the clinician's observations, an assessment and a plan [1]; a GROW note records a mentee's goal, current reality, options and way forward [2]. Documentation extends the working day well beyond the session [3], and ambient AI scribes that draft notes from recorded encounters are now in clinical use and under randomized evaluation [4], [5].

Two gaps limit what such systems can be trusted with. First, a drafted note may contain statements the conversation never supported [6], [7], and the reviewer has no fast way to tell which sentence rests on what. Second, published scribes depend on one model from one vendor, so a quota, an outage, a model retirement or a change in an institution's approved-vendor list stops documentation altogether. Neither gap is addressed for counselling or mentoring conversations, where prior work concentrates on medical visits [8]–[10].

EchoScribe addresses both gaps in one pipeline. Its contributions are:

1) **Evidence-linked notes with cross-vendor verification.** The writer must cite turn numbers for every sentence; a model from a different vendor labels each sentence as supported, partially supported or unsupported; a verification-gated cascade regenerates notes that fail a threshold on another vendor (Section III-C).
2) **An embedded multi-provider router** that serves every processing role from a *combo* of targets with a routing strategy, per-key exponential cooldown, a provider-wide breaker and request-scoped error handling, adapted from the open-source OmniRoute gateway [11] (Section III-A).
3) **Two-party speaker attribution without enrolment**: SepFormer enhancement and ECAPA-TDNN clustering with two fallback paths, followed by role resolution from eight conversation statistics and a model check (Section III-B).
4) **A quantitative evaluation** on public data of the router under injected faults, note quality and grounding on ACI-Bench, verifier accuracy on planted errors, role identification, and speech recognition and speaker attribution under noise (Section IV).

II. Related Work

A. Ambient scribes and note generation

Clinical note generation from doctor–patient dialogue has been benchmarked on ACI-Bench [10] and in the MEDIQA-Chat shared tasks [12], [13]. Adapted language models can match or exceed experts on clinical summarisation [14], and open models have been tuned for expert-level notes with physician reader studies [15]. Evaluation frameworks for ambient scribes now combine computational metrics, reviewer assessment and simulation [8], and deployed systems such as LAOS report recognition and physician review results [9]. For mental health, Adhikary et al. benchmarked language models on summarising counselling sessions [16]. None of these studies produces mentoring (GROW) notes, links each note sentence to its evidence, or remains available when its model vendor fails.

B. Hallucination, grounding and verification

Hallucination is a recurring failure of generated text [6]; Asgari et al. propose a clinical safety framework for annotating hallucinations and omissions in medical summaries [7]. Generic remedies include generating text with citations [17], scoring atomic facts against a source [18], sampling-based self-consistency checks [19] and self-verification plans [20]. EchoScribe combines citation-constrained generation with sentence-level verification by an *independent vendor*, so the checker does not share the writer's training data or failure modes, and uses the verdicts both for the reviewer interface and to trigger regeneration.

C. Speaker diarization, enhancement and recognition

Diarization combines speech activity detection, speaker embeddings and clustering [21]. ECAPA-TDNN embeddings [22] grouped by spectral clustering [23] remain a strong baseline, while end-to-end systems such as pyannote's powerset model [24] and Sortformer [25] learn segmentation directly; DiarizationLM post-corrects speaker labels with a language model [26], and WhisperX aligns recognition with diarization [27]. Zolensky et al. study role identification in clinical conversations [28]. SepFormer [29], distributed in SpeechBrain [30] and trained on WHAM! [31], is one of many enhancement models benchmarked in the DNS challenges [32]; enhancement artefacts can also *hurt* recognition [33], which we test rather than assume.

D. Routing across language models

RouteLLM learns to route queries between a strong and a weak model [34], and FrugalGPT cascades models to reduce cost [35]. Open gateways such as OmniRoute [11] provide combo routing with cooldowns in front of many vendors. EchoScribe embeds this mechanism per processing role inside the application and adds verification as a routing signal: a note is escalated not when a call fails but when its content fails verification.

III. System Design

Fig. 1 shows the pipeline. A browser client records or uploads audio; an Express.js server coordinates every model call; Supabase stores records under per-user row-level security, with therapy and mentoring records separated by a mode column that every query must state. Five roles call language or speech models: STT, DIARIZE, ROLE_LABEL, NOTE_SYNTH and VERIFY.

A. Embedded multi-provider router

Each role is served by a *combo*: an ordered list of targets ⟨vendor, model, keys⟩ and a routing strategy. Vendors are thin adapters exposing `chatCompletion`, `transcribeAudioBuffer` or `diarizeAudioBuffer`; Gemini [36], Groq-hosted gpt-oss-120b [37] and Whisper [38], Deepgram and the local SpeechBrain service ship as adapters. Combos come from the user's settings, from environment variables, or from a reference configuration. The strategy orders targets per request: *priority* (declared order), *round-robin*, *random*, *strict-random* (a shuffled deck), *least-used*, and *lkgp* (the last target that succeeded goes first).

Failures are classified. A key that fails authentication is retired; a rate-limited or failing key enters an exponential cooldown of $5 \cdot 2^{k-1}$ s (capped at 5 min) after its k-th consecutive failure, reset on success; a vendor with 15 failures within 30 min is skipped for 10 min (2 in 5 min for the local service); an HTTP 400 is treated as specific to the request and moves on without cooling the key. These mechanisms and their constants are adapted from OmniRoute [11]. When every target fails, the API returns HTTP 503 with the list of attempts. Keys are kept in the user's browser by default; a user may opt to sync an individual key to the account, where it is stored encrypted with AES-256-GCM and never returned to the client.

B. Speech processing and role resolution

Audio is resampled to 16 kHz mono. When the SpeechBrain service is reachable, SepFormer suppresses noise and ECAPA-TDNN embeddings of 1.5 s windows at a 0.75 s hop are clustered into two speakers by cosine-affinity spectral clustering, with k-means as a fallback when the Laplacian does not separate cleanly. Otherwise DIARIZE falls back to cloud audio diarization, and finally to splitting the recognised transcript into turns with a language model, so a session never collapses to a single speaker. STT transcribes the enhanced audio and the text is distributed over the timed turns.

Roles are resolved without voice enrolment. For each speaker the system computes eight statistics: speaking time, word count and mean turn length, questions, professional vocabulary, guiding phrases, personal and emotional phrases, number of turns, and who spoke first. ROLE_LABEL receives the transcript with these statistics and must assign exactly one professional and one client; invalid answers move to the next target, and a deterministic default is used only if every target fails.

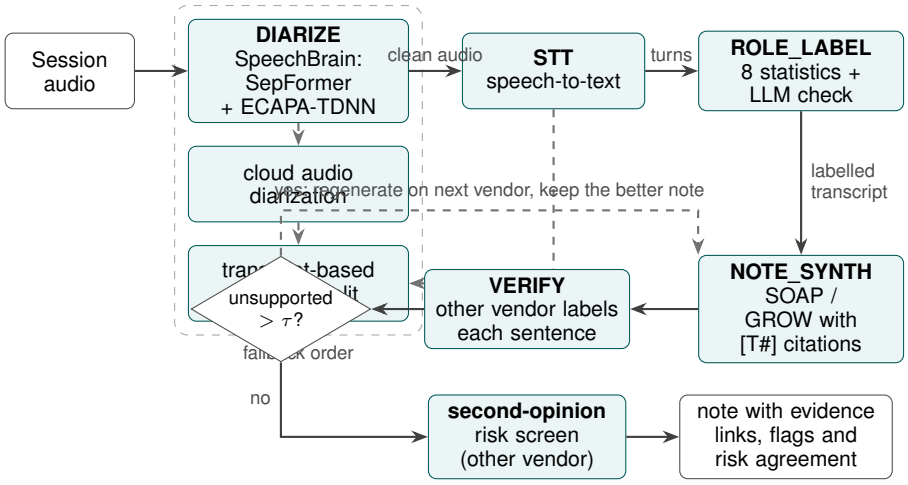


Fig. 1. EchoScribe pipeline. Shaded boxes are processing roles, each served by a combo of the embedded router (Section III-A). Speaker separation falls back from SpeechBrain to cloud audio diarization to a transcript-based split (dashed). The note writer cites transcript turns; a model from another vendor verifies each sentence, and notes over the unsupported threshold τ are regenerated on the next vendor. A second vendor screens independently for risk.

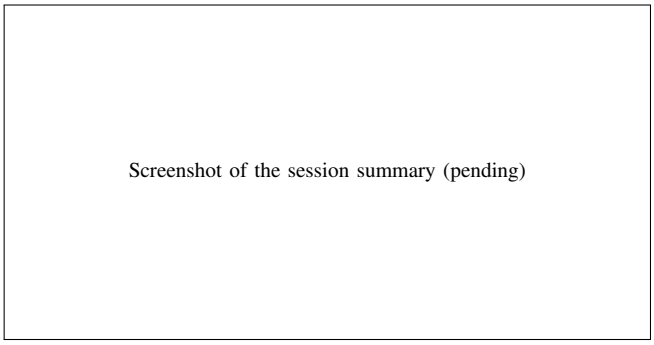


Fig. 2. Session summary in the EchoScribe interface. Hovering a sentence highlights the transcript turns it cites; partially supported and unsupported sentences are marked for review, and the risk panel shows whether the two screening models agree.

C. Evidence-linked notes and cross-vendor verification

The labelled transcript is numbered into turns $T_1 \dots T_n$. NOTE_SYNTH must end every sentence of the note sections, and of the risk notes, with the turns that support it ("…poor sleep for two weeks [T4, T6]"), and must write "Not discussed" rather than an uncited sentence. Citations are parsed into per-sentence evidence and removed from the displayed text. VERIFY then receives each sentence with only its cited turns and labels it *supported*, *partial* (adds a detail the turns lack) or *unsupported*. The router is asked to avoid the writer's vendor, so verification is cross-vendor whenever a second vendor is configured.

If the unsupported fraction exceeds $\tau = 0.2$, the note is regenerated with the writer's vendor moved to the end of the combo, re-verified, and the draft with fewer unsupported sentences is kept. The interface shows each sentence's evidence on hover and marks partial and unsupported sentences for the reviewer (Fig. 2).

D. Risk screen with a second opinion

In therapy mode the writer also screens for suicidal ideation and self-harm; in mentoring mode, for burnout and severe distress. A second vendor repeats the screen independently and cites its evidence turns. The flag is raised if either model raises it, favouring sensitivity, and disagreement is shown as "models disagree — review the transcript". The screen is a model judgement, not a validated instrument such as the C-SSRS or PHQ-9 [39], [40]; language models are known to vary in suicide-risk judgements [41], and the interface states that the clinician remains responsible for risk assessment.

IV. Evaluation

All experiments are scripted in the public repository (`eval/`) and every number below is generated from their result files.

A. Setup

Data. Note generation, verification and role identification use the 40 test dialogues of ACI-Bench (test1, CC BY 4.0) [10], whose doctor–patient visits have reference notes; no public corpus of therapy or mentoring sessions with reference notes exists. Speech experiments use 12 simulated two-person conversations (12.5 min) built from LibriSpeech test-clean [42]: ten alternating utterances from a mixed-gender speaker pair with 0.3–0.9 s gaps, exact reference turns, and three conditions, clean and with babble plus pink noise at 5 dB and 0 dB SNR. **Models.** Gemini models [36] are used in rotation across three keys; gpt-oss-120b [37] and Whisper large-v3 [38] are served by Groq. The model that served each request is logged. **Metrics.** ROUGE [43] and BERTScore [44] against reference notes; WER after text normalisation; DER with a 0.25 s collar [45]; and cpWER, the speaker-attributed WER of CHiME-6 [46], which scores every speaker-separation path including those without timings. Intervals are Wilson 95%.

TABLE I
ROUTER UNDER INJECTED FAULTS (MEAN OF 5 SEEDS): SUCCESS RATE, FAILED PROVIDER CALLS PER REQUEST, AND 95TH-PERCENTILE LATENCY (S).

	Quota			Outage			Mixed		
Policy	Succ.	Fail	p95	Succ.	Fail	p95	Succ.	Fail	p95
No memory	99.97%	0.476	2.4	99.97%	0.291	2.4	99.93%	0.497	31.8
Fixed 60 s	100.00%	0.098	2.3	99.89%	0.087	2.4	98.14%	0.147	31.5
Priority	99.97%	0.227	2.4	99.97%	0.062	2.3	99.86%	0.258	31.7
Round-robin	100.00%	0.014	2.7	99.93%	0.058	2.7	100.00%	0.092	31.4
LKGP	99.97%	0.033	1.9	99.93%	0.040	2.4	100.00%	0.097	9.1

B. Router under injected faults

We drive the router's own code with simulated vendors on a virtual clock: one hour of Poisson traffic at 10 requests/min against a preferred vendor (two keys), a fallback vendor (two keys) and a second fallback, in three scenarios: a per-key quota below the offered load (*quota*); a 15-minute outage of the preferred vendor (*outage*); and a revoked key, a quota and a fallback that times out 20% of the time (*mixed*). We compare walking the chain on every request with no memory, the fixed 60 s suspension of our earlier design, and the router with three strategies (5 seeds each, Table I).

No policy changes success much where a healthy fallback exists, so the differences lie in wasted calls and latency. The last-known-good strategy stops returning to a vendor that keeps timing out and reduces 95th-percentile latency in the mixed scenario from 31.7 s to 9.1 s; round-robin wastes the fewest calls and spreads load most evenly (Jain index [47] 0.90 under quota pressure). Exponential cooldown alone is not uniformly better than a fixed suspension: under a persistent per-minute quota its 5 s first step re-probes an exhausted key (0.227 against 0.098 failed calls per request), while the fixed rule can suspend every option at once and lost requests in the mixed scenario (98.14% success). Priority keeps the clinician's preferred model at the cost of probing; LKGP minimises latency but can stay on a fallback model after the preferred one recovers.

C. Evidence-linked notes and cross-vendor verification

Each of the two writers produced a SOAP note with turn citations for the test dialogues, and the other vendor verified every sentence (Table II). Citation coverage was 95.6% (Gemini) and 97.0% (gpt-oss-120b); 74.6% and 78.6% of sentences were judged fully supported and 5.8% and 7.3% unsupported. With the gate at $\tau = 0.2$, 0 of 24 Gemini-first notes would have been regenerated. ROUGE against ACI-Bench reference notes measures overlap with a differently structured visit note and is reported for comparability, not as a quality ceiling.

Does the verifier catch errors? Model-judged support is only useful if the judge detects real errors. We planted three kinds of error into sentences the verifier had accepted: a changed number, a flipped negation, and a sentence borrowed from another dialogue's note with the original citations; other accepted sentences were kept as controls. Across **??** planted errors the verifier flagged **??** as partial or unsupported, with **??** false alarms on **??** unchanged sentences (Table II, bottom). Controls were sentences this verifier had already accepted, so the false-alarm rate measures consistency rather than precision on fresh sentences.

TABLE II
NOTES ON ACI-BENCH TEST1: OVERLAP WITH REFERENCE NOTES AND CROSS-VENDOR VERIFICATION OF EVERY SENTENCE.

Writer (verifier)	Gemini (gpt-oss)	gpt-oss (Gemini)
Notes	40	24
ROUGE-1 / 2 / L	45.4 / 16.9 / 42.5	45.9 / 14.6 / 41.9
BERTScore F1	**??**	**??**
Sentences per note	18.2	16.6
Sentences with citations	95.6%	97.0%
Supported / partial	74.6% / 6.5%	78.6% / 14.1%
Unsupported	5.8%	7.3%
Notes over gate (τ=0.2)	1	1
Planted errors caught: number	**??**	**??**
negation	**??**	**??**
borrowed sentence	**??**	**??**
False alarms on unchanged	**??**	**??**

TABLE III
SIMULATED TWO-PERSON CONVERSATIONS (12 × 3 CONDITIONS): WER OF SPEECH-TO-TEXT WITH AND WITHOUT SEPFORMER, DER OF SPEECHBRAIN CLUSTERING, AND SPEAKER-ATTRIBUTED CPWER OF EACH SEPARATION PATH (%).

	Clean	5 dB	0 dB
WER Gemini, raw / enhanced	0.0 / 0.0	34.1 / 39.6	71.4 / 51.6
WER Whisper, raw / enhanced	0.0 / 1.1	29.7 / 39.6	60.4 / 74.7
DER SpeechBrain, raw / enh.	22.3 / 22.3	27.2 / 22.3	40.6 / 33.4
cpWER SpeechBrain + STT	33.0	69.2	79.1
cpWER cloud audio diarization	0.0	42.9	51.6
cpWER transcript-based split	0.0	61.5	81.3

D. Role identification

With doctor and patient labels replaced by random speaker identifiers, the eight statistics alone named the clinician correctly in 38 of 40 dialogues (95.0%), the model alone in 37 (92.5%), and their combination, as deployed, in 39 (97.5%). Only 2 dialogues had the patient speaking more than the doctor, too few to test the case the statistics are meant for; this is part of the role-play protocol below.

E. Speech recognition and speaker attribution

Table III reports the speech results. **[Interpretation to be written when the audio run completes.]**

Processing cost per minute of audio on a laptop CPU was 57.8 s for SepFormer enhancement with diarization (windowed, bounded memory), 7.9 s for diarization alone, 1.4 s for Whisper on Groq and 29.5 s for Gemini speech-to-text.

V. DISCUSSION AND LIMITATIONS

What the results support. Citation-constrained writing kept almost every sentence attached to evidence, and an independent vendor caught most planted factual errors, so sentence-level flags give a reviewer a concrete place to look

instead of an unexplained confidence score [48]. The router results show that resilience comes from remembering failures, and that the choice of strategy is a trade-off between staying on the preferred model and minimising latency, not a single best setting. Role identification without enrolment was accurate on two-party clinical dialogue, and the combination of statistics and model was more accurate than either alone.

Deployment. EchoScribe runs as a local application: the server, the speech service and the database client run on the clinician's machine, and only the configured model vendors receive audio or text. Keys stay in the browser unless the user chooses to sync one. Sessions are not limited in length; long recordings are compressed to mono Opus before they reach a cloud model, enhancement runs in bounded memory, and timeouts scale with duration. Because model access is per role and per vendor, an institution can restrict processing to vendors its data-governance policy approves, or to the local SpeechBrain service for speaker separation.

Limitations. (i) ACI-Bench contains medical visits, not therapy or mentoring sessions; no public corpus of counselling sessions with reference notes exists, and GROW notes were not evaluated quantitatively. (ii) Support labels come from a model; the planted-error study bounds how often it misses errors of three kinds, but subtler omissions and misattributions need human review [7]. (iii) Speech results come from simulated conversations of read speech without overlap; spontaneous, overlapping clinical speech will be harder. (iv) Results depend on specific model versions that vendors retire; we log the serving model of every request. (v) The risk screen has not been validated against clinician labels. The protocol released with the code covers these gaps: blinded ratings of evidence-linked against single-pass notes by clinicians and mentors, annotated role-played sessions for real-speech WER, DER and role accuracy, and a clinician-labelled vignette probe of the risk screen.

VI. Conclusion

EchoScribe produces SOAP and GROW notes from recorded two-person sessions in which every sentence points to the turns that support it and is checked by a model from a different vendor, with a verification-gated cascade, a two-model risk screen that surfaces disagreement, and an embedded router that keeps every processing role available through quota exhaustion, outages and revoked keys. On public data the notes were almost fully cited, the cross-vendor verifier caught most planted errors, role identification needed no enrolment, and the router's last-known-good strategy cut tail latency under compound faults. The code, evaluation scripts and human-evaluation protocol are available at https://github.com/Aravind-Krishnan-S/ECHOSCRIBE.

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